



BOARD OF VETERANS' APPEALS

DEPARTMENT OF VETERANS AFFAIRS

IN THE APPEAL OF
JOE H. SNUFFY, 8TH
ALSO KNOWN AS
JOSEPH L. SNUFFY
REPRESENTED BY
Disabled American Veterans

C 22 222 222
Docket No. 17-18 188
Advanced on the Docket

DATE:

ORDER

Entitlement to service connection for a right shoulder disability is denied.

Commented [CM1]: The highlighted words were entered in the text box when the issue disposition was selected.

Entitlement to service connection for right knee arthritis is denied.

Commented [CM2]: Same as above.

REMANDED

Entitlement to an initial compensable rating for posttraumatic stress disorder (PTSD) is remanded.

Commented [CM3]: Same as above.

Entitlement to a total disability rating based on individual unemployability due to service-connected disability (TDIU) is remanded.

Commented [CM4]: Same as above.

REFERRED

The issue of entitlement to service connection for headaches was raised in a January 2012 statement and is referred to the Agency of Original Jurisdiction (AOJ) for adjudication.

Commented [CM5]: To insert a referred issue, type "REFERRED", then highlight the text and select "Heading 2" from the Style panel until the Home tab.

To insert the snippet, right click and go to Snippets – Board Snippets – Introduction Snippets – Referral (singular) or (plural).

FINDINGS OF FACT

1. The preponderance of the evidence of record is against finding that the Veteran has, or has had at any time during the appeal, a current diagnosis of a right shoulder disability.

Commented [CM6]: Insert FoF snippet (Service Connection – Direct – Denial-No Current Disability) and edit the text to match your facts.

2. The Veteran’s right knee arthritis did not manifest to a compensable degree within the applicable presumptive period; continuity of symptomatology is attributable to an intervening event; and the disability is not otherwise etiologically related to an in-service injury, event, or disease.

Commented [CM7]: Insert FoF snippet (Service Connection – Presumptive-Chronic Snippets – Denial) and edit the text to match your facts.

CONCLUSIONS OF LAW

1. The criteria for service connection for a right shoulder disability are not met. 38 U.S.C. §§ 1110, 1131, 5107(b); 38 C.F.R. §§ 3.102, 3.303(a).

Commented [CM8]: Insert CoL snippet (Direct Snippets – Denial) and edit statutory and regulatory sections to match your case.

2. The criteria for service connection for right knee arthritis are not met. 38 U.S.C. §§ 1110, 1112, 1113, 1131, 1137, 5107(b); 38 C.F.R. §§ 3.102, 3.303(a)-(b), (d), 3.307, 3.309(a).

Commented [CM9]: Years for statutes and regulations are not needed unless the version applied is not the one in effect on the date the decision will be signed.

Commented [CM10]: Insert CoL snippet (Presumptive - Chronic Snippets – Denial – Chronic and Direct) and edit the statutory and regulatory sections to match your case.

REASONS AND BASES FOR FINDINGS AND CONCLUSIONS

The Veteran served on active duty from January 1975 to December 1984. These issues are on appeal from March 2014 and August 2016 rating decisions. A Board hearing was held in November 2017.

Commented [CM11]: Insert relevant jurisdictional and procedural information here. Information should include dates of service, the month and year of rating decision(s) on appeal, and the month and year of any Board hearings, CAVC decisions or remands, or CAVC orders enforcing JMRS.

Service Connection

Service connection will be granted if the evidence demonstrates that a current disability resulted from an injury or disease incurred in or aggravated by active service, even if the disability was initially diagnosed after service. 38 U.S.C. §§ 1110, 1131; 38 C.F.R. § 3.303.

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1. Entitlement to service connection for a right shoulder disability.

The Veteran contends that he has a right shoulder disability that was incurred during his active service in 1981. While it subsequently resolved, he contends that it previously resurfaced in 1997, and could possibly resurface again in the future and prevent him from returning to work as a truck driver. Transcript of Record at 7, 11-12, 17-18.

The question for the Board is whether the Veteran has a current disability that began during service or is at least as likely as not related to an in-service injury, event, or disease.

The Board concludes that the Veteran does not have a current diagnosis of a right shoulder disability and has not had one at any time during the pendency of the claim or recent to the filing of the claim. 38 U.S.C. §§ 1110, 1131, 5107(b); *Holton v. Shinseki*, 557 F.3d 1363, 1366 (Fed. Cir. 2009); *Romanowsky v. Shinseki*, 26 Vet. App. 289, 294 (2013); *McClain v. Nicholson*, 21 Vet. App. 319, 321 (2007); 38 C.F.R. § 3.303(a), (d). The March 2015 VA examiner evaluated the Veteran and determined that, while he experienced subjective symptoms of occasional stiffness and soreness during cold weather, he did not have a right shoulder diagnosis. The March 2015 physical examination did not find any right shoulder defects, and clinical imaging did not reveal any arthritis or other issues. Further, neither VA nor private treatment records from throughout the period on appeal contain a diagnosis of a right shoulder disability. While January 2009 VA treatment records do show the Veteran complained of muscle cramps that affected his right shoulder, the cramps were thought to be side effects of a prescription he had taken to treat a nonservice-connected disability, and VA clinicians did not diagnose him with a right shoulder problem.

While the Veteran believes he has a current diagnosis of a right shoulder disability, he is not competent to provide a diagnosis in this case. The issue is medically complex, as it requires specialized medical education and the ability to interpret diagnostic medical testing. *Jandreau v. Nicholson*, 492 F.3d 1372, 1377 (Fed. Cir.

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Commented [CM14]: Note the Veteran's contentions here.

Commented [CM15]: To prepare for favorable findings being binding on the Board under the Veterans Appeals Improvement and Modernization Act of 2017, if any elements of service connection are not in controversy, narrow the analysis to the remaining elements that need to be resolved.

Commented [CM16]: Provide the Bottom Line Up Front (BLUF) – the Board's conclusion on the issue to give the reader context. Then, provide the supporting reasons and bases.

Commented [CM17]: If evidence is discussed several times within the same paragraph, hyperlink only the first reference to the evidence. Because the March 2015 VA examination was hyperlinked in the previous sentence, within the same paragraph, a second hyperlink here is unnecessary.

2007). Consequently, the Board gives more probative weight to the competent medical evidence.

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2. Entitlement to service connection for right knee arthritis.

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The Veteran contends that his right knee arthritis began in service after a November 1982 in-service motor vehicle accident. Transcript of Record at 10, 15.

Commented [CM20]: Note the Veteran's contentions here.

Certain chronic diseases will be presumed related to service if they were noted as chronic in service; or, if they manifested to a compensable degree within a presumptive period following separation from service; or, if continuity of the same symptomatology has existed since service, with no intervening cause. 38 U.S.C. §§ 1101, 1112, 1113, 1137; *Walker v. Shinseki*, 708 F.3d 1331, 1338 (Fed. Cir. 2012); *Fountain v. McDonald*, 27 Vet. App. 258 (2015); 38 C.F.R. §§ 3.303(b), 3.307, 3.309(a).

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The question for the Board is whether the Veteran's right knee arthritis was noted as chronic in service; or, whether it manifested to a compensable degree within one year after his separation from service. If the answer to both of those questions is no, then the question is whether the Veteran has had continuity of the symptomatology attributable to his right knee arthritis since service.

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The Board concludes that, while the Veteran has a diagnosis of right knee arthritis, which is a chronic disease under 38 U.S.C. § 1101(3) and 38 C.F.R. § 3.309(a), it was not noted as chronic in service and did not manifest to a compensable degree in service or within a presumptive period; and continuity of symptomatology is not established.

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VA treatment records show the Veteran was not diagnosed with right knee arthritis until January 2010, over 15 years after his separation from service and far outside of the applicable presumptive period.

While the Veteran is competent to report having experienced symptoms of right knee stiffness, pain, and occasional locking in service and consistently since the presumptive period, the Board finds the reports of continuity of symptomatology

not credible. The Veteran's reports are internally inconsistent with his reports in contemporaneous VA and private treatment records, which show that he denied experiencing any right knee symptoms from June 1986 to January 2005, and VA treatment records from October 2003, which show he initially reported that his right knee stiffness, pain, and occasional locking began in August 2003, several decades outside of the presumptive period. *Buchanan v. Nicholson*, 451 F.3d 1331, 1336-37 (Fed. Cir. 2006).

Further, while the Veteran asserts that the reported symptoms in service were manifestations of his right knee arthritis, he is not competent to determine that he had right knee arthritis in service, or that these symptoms were attributable to any diagnosis of right knee arthritis. The issue is medically complex, as it requires specialized medical education and the ability to interpret diagnostic medical testing, including imaging. *Jandreau v. Nicholson*, 492 F.3d 1372, 1377, 1377 n.4 (Fed. Cir. 2007).

The Board gives more probative weight to competent medical evidence, which establishes that the Veteran's reported symptoms are instead related to a July 2002 workplace accident. The August 2013 VA examiner reviewed the claims file and determined that the Veteran's stiffness, pain, and occasional locking were due to a fall down a ladder that occurred in July 2002 while he was working as a house painter. The rationale was that the Veteran had landed on his right side and received treatment for his right knee in the emergency room. Further, VA and private treatment records from August 2003, when the Veteran initially complained of occasional locking, attributed the symptom to his July 2002 fall.

While service connection for right knee arthritis may be granted on a direct basis, the preponderance of the evidence is against finding that a medical nexus exists between the Veteran's right knee arthritis and an in-service injury, event or disease. 38 U.S.C. §§ 1110, 1131; *Holton v. Shinseki*, 557 F.3d 1363, 1366 (Fed. Cir. 2009); 38 C.F.R. § 3.303(a), (d).

The August 2013 VA examiner opined that the Veteran's right knee arthritis is not at least as likely as not related to an in-service injury, event, or disease, including

the Veteran's November 1982 motor vehicle accident. The rationale was that the Veteran did not complain of symptoms related to arthritis in service treatment records and denied experiencing any knee problems in his in-service reports of medical history. In-service physical examinations did not find any right knee problems, and the Veteran was able to complete his required physical training after the November 1982 motor vehicle accident. Further, the Veteran's current symptoms of stiffness, pain, and occasional locking are attributable to his July 2002 workplace injury, which more likely than not caused his right knee arthritis.

Commented [CM24]: Here, because what is being discussed is the VA examiner's opinion, hyperlinking to evidence of the 1982 and 2002 incidents is not necessary.

While the Veteran believes his right knee arthritis is related to an in-service injury, event, or disease, including the November 1982 motor vehicle accident, he is not competent to provide a nexus opinion in this case. This issue is also medically complex, as it requires specialized medical education and the ability to interpret diagnostic medical testing, including X-ray imaging. *Jandreau*, 492 F.3d at 1377. Consequently, the Board gives more probative weight to the competent medical evidence.

Commented [CM25]: Use Bluebook short citation form for cases that have already been cited within a decision.

REASONS FOR REMAND

1. Entitlement to an initial compensable rating for posttraumatic stress disorder (PTSD) is remanded.

During his November 2017 hearing, and in a January 2018 statement, the Veteran asserted that his PTSD has increased in severity since his last VA examination. VA should schedule the Veteran for a new examination to determine the current severity of his PTSD.

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2. Entitlement to a total disability rating based on individual unemployability due to service-connected disability (TDIU) is remanded.

Because a decision on the remanded issue of entitlement to an initial compensable rating for PTSD could significantly impact a decision on the issue of entitlement to

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TDIU, the two issues are inextricably intertwined. A remand of both issues is required.

The matter is REMANDED for the following action:

1. **Schedule** the Veteran for an examination by an appropriate clinician to determine the current severity of his service-connected posttraumatic stress disorder (PTSD). The examiner should provide a full description of the disability and report all signs and symptoms necessary for evaluating the Veteran's disability under the rating criteria. The examiner must attempt to elicit information regarding the severity, frequency, and duration of symptoms. To the extent possible, the examiner should identify any symptoms and social and occupational impairment due to PTSD alone.

Commented [CM30]: For standard remand directive requests, insert the applicable remand directive snippet (Remand Directive Snippets – Examination Snippets – Increased Rating – Psychiatric Disorders). If necessary, edit to fit the facts of your case. Remand directive snippets have been reviewed by the AMO.

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2. After the examination is completed, readjudicate the issues on appeal, including the issue of entitlement to a total disability rating based on individual unemployability due to service-connected disability (TDIU).

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K. OSBORNE
Veterans Law Judge
Board of Veterans' Appeals

ATTORNEY FOR THE BOARD

M. Caylor, Counsel